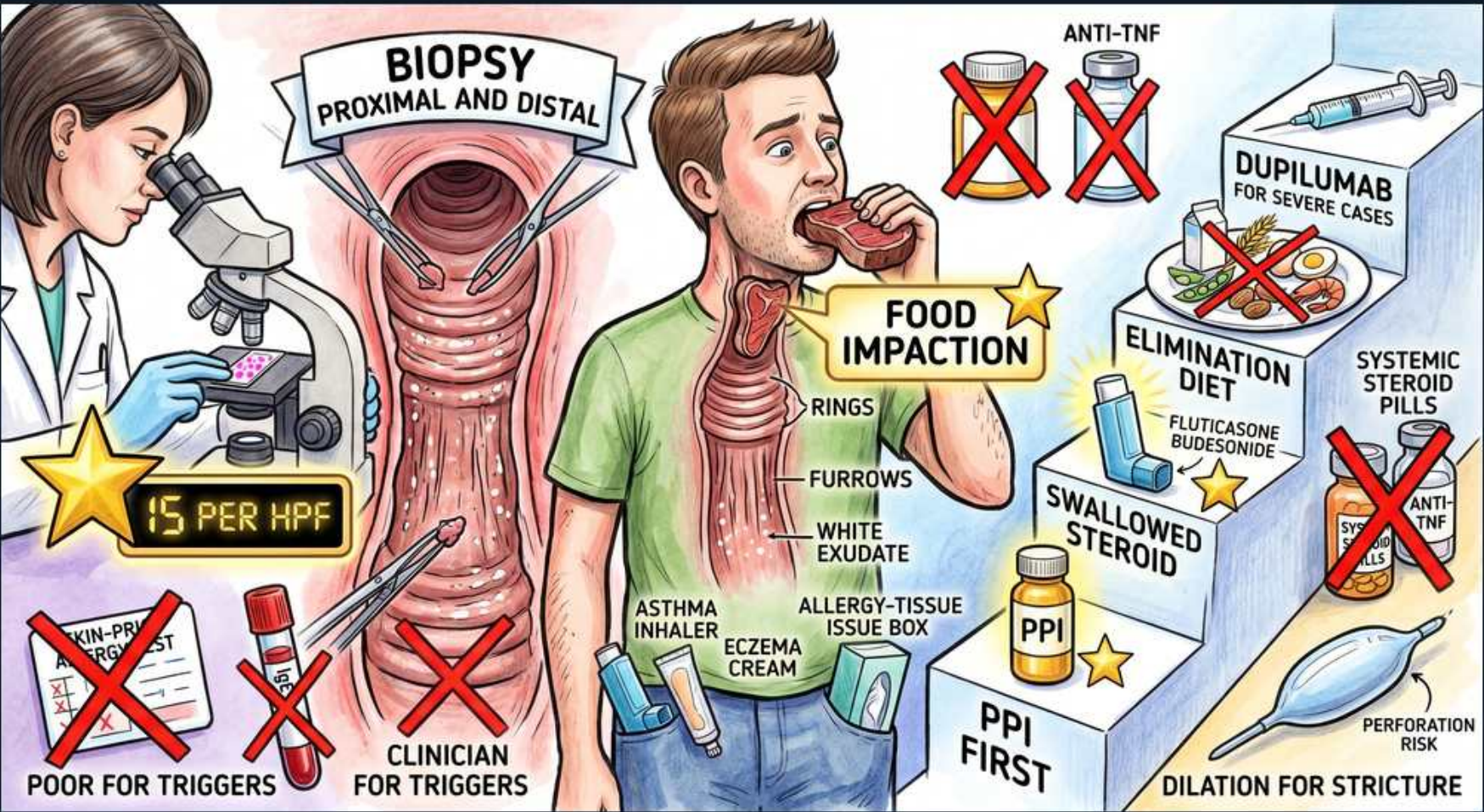


Esophageal — Eosinophilic Esophagitis



1. Young atopic male, food impaction

WHAT YOU SEE

Young man biting food with a gold-star 'FOOD IMPACTION' label and a stuck bolus.

WHAT IT ENCODES

Typical EoE: young/middle-aged male with a history of atopy (asthma, eczema, allergic rhinitis, food allergy) presenting with dysphagia to solids and recurrent FOOD IMPACTION. Food bolus impaction in a young man should trigger EoE workup.

2. Atopy: asthma & eczema

WHAT YOU SEE

An 'ASTHMA INHALER' and 'ECZEMA CREAM' icons by the patient.

WHAT IT ENCODES

EoE is a Th2/allergic, antigen-driven disease; the patient usually has the atopic triad (asthma, atopic dermatitis, allergic rhinitis) and food allergies. This atopic background is a key history clue distinguishing it from GERD.

3. Rings

WHAT YOU SEE

Endoscopy view labeled 'RINGS' (concentric trachealization).

WHAT IT ENCODES

Endoscopic findings: concentric mucosal RINGS (trachealization/feline esophagus), longitudinal furrows, white exudates/plaques, edema, and strictures with narrow-caliber esophagus. These suggest but do not confirm EoE.

4. Furrows

WHAT YOU SEE

Endoscopy label 'FURROWS' (longitudinal grooves).

WHAT IT ENCODES

Linear longitudinal furrows reflect mucosal edema/eosinophilic infiltration. Along with rings and white exudates, they form the classic endoscopic triad, but biopsy is still required for diagnosis.

5. White exudate

WHAT YOU SEE

Endoscopy label 'WHITE EXUDATE' (whitish plaques).

WHAT IT ENCODES

White exudates/plaques are eosinophilic microabscesses (can mimic Candida). Their presence in an atopic young man with dysphagia points to EoE; confirm with biopsy showing dense eosinophilia.

6. Biopsy proximal AND distal (≥ 15 eos/HPF)

WHAT YOU SEE

Pathologist at a microscope, banner 'BIOPSY PROXIMAL AND DISTAL', gold-star '15 PER HPF'.

WHAT IT ENCODES

Diagnosis requires esophageal BIOPSY from BOTH proximal and distal esophagus (≥ 2 -4 each) showing ≥ 15 eosinophils per high-power field, after a PPI trial, plus exclusion of other causes. Sampling multiple levels avoids missing patchy disease.

7. Diagnose by appearance (RED X)

WHAT YOU SEE

Endoscope icon with a red X near the top.

WHAT IT ENCODES

Diagnosing EoE from endoscopic appearance (rings/furrows) ALONE is wrong; the esophagus can look normal and findings overlap with GERD. Histology with ≥ 15 eos/HPF from proximal and distal biopsies is mandatory.

BOARD TRAP

'diagnose EoE by endoscopic appearance without biopsy' -> wrong; need ≥ 15 eos/HPF on proximal AND distal biopsy.

8. Skin-prick / serum IgE (RED X)

WHAT YOU SEE

'SKIN-PRICK / ALLERGY' and serum-tube icons crossed out, 'POOR FOR TRIGGERS'.

WHAT IT ENCODES

Allergy testing (skin-prick, serum specific IgE) is POOR at identifying EoE food triggers and is not used to guide therapy. Empiric elimination diet (e.g., six-food or step-up) guided by repeat biopsy is preferred over allergy-test-directed diets.

BOARD TRAP

'use skin-prick / serum IgE to find EoE triggers' -> wrong; allergy testing is unreliable here; use empiric elimination diet.

9. PPI first (gold star)

WHAT YOU SEE

Block step 'PPI' with a gold star and 'PPI FIRST' banner.

WHAT IT ENCODES

First-line therapy: a PPI (high-dose, twice daily) is now considered a treatment for EoE (not just to exclude GERD); many patients achieve histologic remission. Step up to topical steroid or diet if no response. PPI is the starting rung of the ladder.

10. Swallowed topical steroid (gold star)

WHAT YOU SEE

Step 'SWALLOWED STEROID' gold star, with 'FLUTICASONE / BUDESONIDE'.

WHAT IT ENCODES

Next rung: SWALLOWED (topical) corticosteroid, fluticasone from an inhaler swallowed (not inhaled) or oral viscous budesonide; do not eat/drink for ~30 min after. Watch for esophageal candidiasis. This is the steroid form used in EoE, not pills.

11. Elimination diet

WHAT YOU SEE

Step 'ELIMINATION DIET' with a crossed-out food plate.

WHAT IT ENCODES

Empiric food elimination (commonly dairy, wheat, egg, soy/legumes, nuts, seafood = six-food, or step-up 2-4-6) with serial biopsies to identify triggers. An effective dietary alternative to drugs, especially for patients preferring no medication.

12. Dupilumab for severe

WHAT YOU SEE

Top step 'DUPILUMAB FOR SEVERE CASES'.

WHAT IT ENCODES

Dupilumab (anti-IL-4Ralpha, blocks IL-4/IL-13) is approved for refractory/severe EoE not controlled by PPI/topical steroid/diet. It is the biologic option at the top of the ladder.

13. Systemic steroid pills / anti-TNF (RED X)

WHAT YOU SEE

'SYSTEMIC STEROID PILLS' and 'ANTI-TNF' icons crossed out with red X.

WHAT IT ENCODES

Systemic (oral/IV) corticosteroids and anti-TNF agents are NOT standard EoE therapy; systemic steroids carry needless toxicity versus equally effective swallowed topical steroids, and anti-TNF is ineffective. Use topical steroid, not systemic.

BOARD TRAP

'systemic steroid pills or anti-TNF for EoE' -> wrong; use swallowed topical steroid; systemic steroids add toxicity and anti-TNF doesn't work.

14. Dilation for strictures (perforation risk)

WHAT YOU SEE

Balloon-dilation icon labeled 'DILATION FOR STRICTURE / PERFORATION RISK'.

WHAT IT ENCODES

Esophageal dilation relieves fibrostenotic strictures/narrow-caliber esophagus causing dysphagia but does not treat the underlying inflammation and carries perforation/mucosal-tear and post-procedure chest pain risk. Combine with medical/diet therapy.